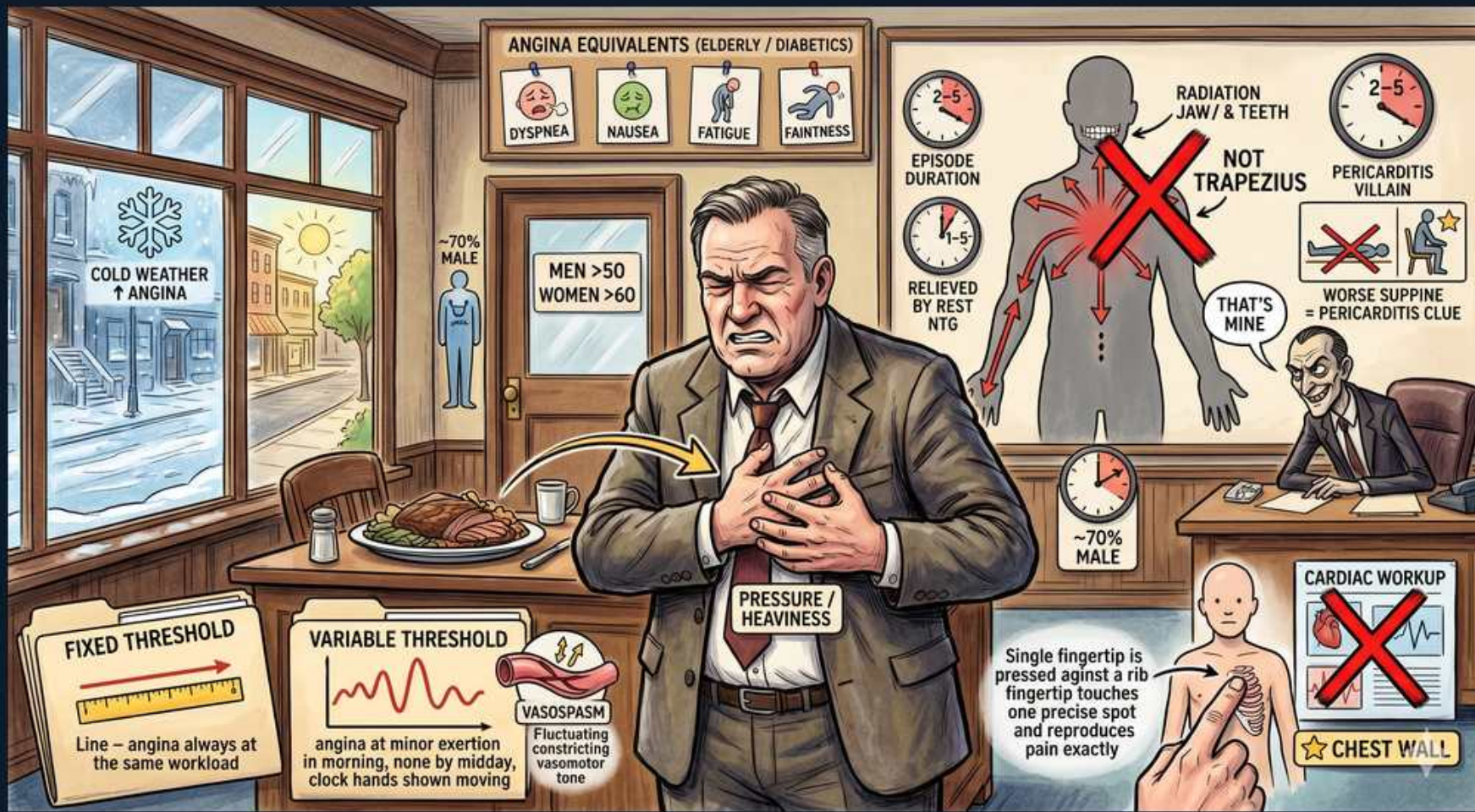


Angina — History, Equivalents & Atypical Mimics



1. Pressure / heaviness — quality

WHAT YOU SEE

Man clutching chest, 'PRESSURE / HEAVINESS' tag.

WHAT IT ENCODES

Typical angina is described as pressure, heaviness, tightness, or squeezing — not sharp or stabbing. It is substernal, provoked by exertion/stress, and relieved by rest or nitroglycerin. Three features define typical angina.

BOARD TRAP

Sharp, stabbing, pinpoint pain argues AGAINST cardiac ischemia — anginal pain is dull pressure, poorly localized.

2. Angina equivalents (elderly/diabetics)

WHAT YOU SEE

Banner 'ANGINA EQUIVALENTS (ELDERLY / DIABETICS)': dyspnea, nausea, fatigue, faintness.

WHAT IT ENCODES

Elderly, diabetic, and female patients may have NO chest pain — instead dyspnea, nausea, fatigue, diaphoresis, or syncope (anginal equivalents). Diabetic autonomic neuropathy blunts pain perception (silent ischemia).

BOARD TRAP

Absence of chest pain does NOT exclude ACS in diabetics/elderly — dyspnea or fatigue may be the only clue (silent MI).

3. Radiation jaw/teeth/arm

WHAT YOU SEE

Body diagram 'RADIATION JAW & TEETH', arm radiation arrows.

WHAT IT ENCODES

Anginal pain classically radiates to the left arm, both arms, jaw, neck, or teeth. Radiation to both arms or to the right arm raises the likelihood of true ischemia significantly.

BOARD TRAP

Pain radiating to the back/jaw/arms supports ischemia; pain localized to one fingertip on the chest wall does not.

4. NOT trapezius — pericarditis clue

WHAT YOU SEE

'NOT TRAPEZIUS' label with red X over trapezius radiation.

WHAT IT ENCODES

Pain radiating to the trapezius ridge is characteristic of PERICARDITIS (phrenic nerve), not myocardial ischemia. This is a key discriminator pointing away from angina.

BOARD TRAP

Trapezius-ridge radiation = pericarditis, not angina — a classic board distinguisher.

5. Worse supine — pericarditis clue

WHAT YOU SEE

'WORSE SUPINE = PERICARDITIS CLUE' near the villain figure.

WHAT IT ENCODES

Pericarditic pain is pleuritic, worse lying flat (supine), and relieved by sitting forward. Anginal pain is not positional. These positional/pleuritic features separate pericarditis from ischemia.

BOARD TRAP

Positional, pleuritic pain worse when supine and better leaning forward = pericarditis — not exertional angina.

6. Episode duration — minutes vs prolonged

WHAT YOU SEE

Clock 'EPISODE DURATION'.

WHAT IT ENCODES

Stable angina lasts minutes (typically <5-10 min) and resolves with rest/nitrates. Pain lasting seconds or for many hours/days is unlikely ischemic; prolonged unremitting pain suggests MI.

BOARD TRAP

Pain lasting only seconds or persisting for days is NOT typical angina — duration of minutes relieved by rest is the pattern.

7. Relieved by rest / NTG

WHAT YOU SEE

'RELIEVED BY REST + NTG' clock icon.

WHAT IT ENCODES

Relief with rest or sublingual nitroglycerin within minutes supports stable angina. Note esophageal spasm also responds to nitrates, so NTG relief is suggestive but not diagnostic.

BOARD TRAP

Nitroglycerin relief is NOT proof of cardiac origin — esophageal spasm also responds to NTG.

8. Fixed threshold — stable angina

WHAT YOU SEE

'FIXED THRESHOLD' panel, 'angina always at the same workload'.

WHAT IT ENCODES

Stable angina has a fixed, reproducible threshold — symptoms appear at a predictable level of exertion and resolve with rest. A stable demand-supply mismatch from fixed coronary stenosis.

BOARD TRAP

A reproducible, exertion-linked threshold = stable angina; pain at rest or with a falling threshold signals unstable angina/ACS.

9. Variable threshold / vasospasm

WHAT YOU SEE

'VARIABLE THRESHOLD' panel, 'angina at minor exertion in morning', 'VASOSPASM'.

WHAT IT ENCODES

A variable threshold (worse in cold/morning, with emotional stress) suggests a vasospastic component (Prinzmetal/vasospastic angina) superimposed on demand ischemia. Triggered by transient coronary tone.

BOARD TRAP

Morning/cold-triggered, rest-occurring angina with transient ST elevation = vasospastic (Prinzmetal) angina — treat with CCB/nitrates, NOT beta-blockers alone.

10. Cold weather — increased angina

WHAT YOU SEE

Snowflake window 'COLD WEATHER ↑ ANGINA'.

WHAT IT ENCODES

Cold exposure raises afterload and coronary tone, lowering the anginal threshold. Cold-induced angina supports a vasospastic/demand contribution and is a recognized trigger.

BOARD TRAP

Cold-triggered angina points toward vasospasm or increased demand — a clue, not a benign feature to ignore.

11. Age/sex risk — men >50, women >60

WHAT YOU SEE

'MEN >50 / WOMEN >60' and '~70% MALE' figures.

WHAT IT ENCODES

Pretest probability of CAD rises with age and male sex; women typically present ~10 years later (post-menopause). Demographics shape how aggressively to pursue ischemic workup.

BOARD TRAP

Atypical symptoms in a low-pretest-probability young woman shouldn't be reflexively called ACS — but never dismiss diabetics/post-menopausal women.

12. Reproducible chest-wall pain — not cardiac

WHAT YOU SEE

'Single fingertip reproduces pain exactly' / 'CARDIAC WORKUP' crossed out, CHEST WALL star.

WHAT IT ENCODES

Pain reproduced exactly by single-fingertip palpation of one spot is musculoskeletal (costochondritis/chest-wall), making ischemia unlikely. Reproducible positional/palpable pain lowers cardiac probability.

BOARD TRAP

Point-tender, palpation-reproducible chest pain is musculoskeletal — but it never fully excludes ACS in a high-risk patient; weigh the whole picture.
